

TMS LIST

FOR CLINICAL DIRECTORS & QUALITY MANAGERS

The TMS Patient Outcome Tracking System

A clinical framework for measuring, reporting, and improving Transcranial Magnetic Stimulation treatment outcomes — from baseline assessment through 6-month follow-up.

What this guide covers:

- Standardized outcome measures & validated instruments
- Assessment schedules, tracking spreadsheets & tabs
- Response and remission criteria definitions
- Adverse event classification and documentation
- Quality improvement protocols and aggregate reporting

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TMS Patient Outcome Tracking System

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Implementing standardized outcome tracking transformed how we communicate with payers and referring physicians. We can now demonstrate remission rates in peer-to-peer reviews — which has meaningfully improved authorization approvals.

Clinical Director — Multi-Site TMS Practice, Northeast US | 9 years TMS experience

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The PHQ-9 trajectory data alone gave us the evidence to adjust our protocols for partial responders. Our response rates improved by 18% within two quarters of adopting this framework.

Quality Manager — Academic Medical Center TMS Program, Midwest | 4 years TMS experience

Why Track Outcomes?

Systematic outcome measurement is the cornerstone of high-quality TMS practice. Beyond clinical necessity, outcome data increasingly drives payer relations, regulatory compliance, and the advancement of the TMS evidence base.



Clinical Excellence

Identify non-responders at Week 4 and adjust stimulation parameters before the authorized course is complete. Early identification of partial response enables timely protocol modifications — changing coil placement, increasing intensity, or adjusting pulse frequency.



Insurance & Payer Relations

Aggregate outcome data supports continued treatment authorization and simplifies peer-to-peer reviews. A documented response trajectory gives medical directors clear clinical justification for ongoing care — reducing administrative friction and denials.



Referring Physician Marketing

Shareable outcome summaries (de-identified aggregate data) demonstrate practice quality to referring psychiatrists, primary care physicians, and pain specialists. This builds referral volume and strengthens professional network relationships.



Research & Publications

Consistent data collection across patients positions your practice to contribute to the TMS evidence base. Multi-site collaborations and journal submissions require standardized outcome measures — the data must be collected systematically to be usable.



Regulatory & FDA Post-Market Surveillance

FDA clearance for TMS devices carries post-market surveillance obligations. Documented adverse events and outcome measures support compliance reporting requirements and demonstrate ongoing device safety monitoring as part of your quality management system.

Core Outcome Measures

PHQ-9

PRIMARY MEASURE

DEPRESSION

Items: 9 **Score range:** 0 – 27 **Clinically significant improvement:** 5+ point decrease from baseline**Remission threshold:** Score < 5

Validated self-report tool for depression severity. Administered at baseline, Week 4, Week 6, post-treatment, and all follow-up points. Change score (baseline minus follow-up) is the primary efficacy endpoint for most TMS clinical protocols.

GAD-7

ANXIETY

Items: 7 **Score range:** 0 – 21 **Clinically significant improvement:** 4+ point decrease from baseline**Remission threshold:** Score < 5

Generalized Anxiety Disorder 7-item scale. Often comorbid with depression in TMS patients. Capturing GAD-7 data alongside PHQ-9 provides a fuller clinical picture and supports treatment of anxiety-spectrum presentations.

CGI-S

CLINICIAN-RATED

BASELINE ONLY

Scale: 1 – 7 **Baseline:** Required at Week 0 (pre-treatment)**Post-treatment:** Required at post-treatment assessment **Follow-up:** Required at 1mo, 3mo, 6mo

Clinical Global Impression – Severity. Clinician-rated global assessment of illness severity. Score of 1 = normal, 4 = moderately ill, 7 = among the most extremely ill patients. Provides an independent anchor for self-reported PHQ-9 and GAD-7 scores.

CGI-I

CLINICIAN-RATED

Scale: 1 – 7 **Timing:** Week 2, Week 4, Week 6, post-treatment

Interpretation: 1 = very much improved; 2 = much improved; 3 = minimally improved; 4 = no change; 5 = minimally worse; 6 = much worse; 7 = very much worse

Clinical Global Impression – Improvement. Clinician-rated global assessment of change from baseline. A CGI-I score of 1 or 2 (much or very much improved) combined with PHQ-9 improvement defines clinical response.

EQ-5D-5L

Dimensions: 5

Timing: Baseline and post-treatment

EuroQol quality-of-life instrument measuring mobility, self-care, usual activities, pain/discomfort, and anxiety/depression. Generates a health utility index used in health economics analyses and payer negotiations.

Patient Satisfaction Survey

Items: 10-item Likert scale

Timing: Baseline (expectations) and post-treatment

Custom clinic-developed instrument capturing treatment experience, provider communication, scheduling convenience, perceived benefit, and likelihood to recommend. Compares pre- and post-treatment expectations vs. experience.

Assessment Schedule

The following schedule represents a clinically validated assessment timeline for standard TMS depression protocols. All measures should be administered at the scheduled timepoint regardless of interim clinical impressions — objective data collection must be consistent to be meaningful.

Timepoint	PHQ-9	GAD-7	CGI-S	CGI-I	EQ-5D-5L	Satis
Baseline (Week 0)	REQUIRED	REQUIRED	REQUIRED	N/A	REQUIRED	REQ
Week 2	OPTIONAL	OPTIONAL	N/A	REQUIRED	N/A	N/A
Week 4	REQUIRED	REQUIRED	N/A	REQUIRED	N/A	N/A
Week 6	REQUIRED	REQUIRED	N/A	REQUIRED	N/A	N/A
Post-Tx (Wk 7–9)	REQUIRED	REQUIRED	REQUIRED	REQUIRED	REQUIRED	REQ
1-Month Follow-Up	REQUIRED	REQUIRED	REQUIRED	N/A	N/A	OPT
3-Month Follow-Up	REQUIRED	REQUIRED	REQUIRED	N/A	N/A	OPT
6-Month Follow-Up	REQUIRED	REQUIRED	REQUIRED	N/A	N/A	OPT

Note on scheduling flexibility: The post-treatment window of Week 7–9 accommodates scheduling variations. Record the actual assessment date in the tracking spreadsheet — do not shift other timepoints to compensate. The 1-, 3-, and 6-month follow-up windows are measured from the *last treatment session*, not the post-treatment assessment date.

Week 4 is the critical decision point. If a patient shows less than 25% PHQ-9 improvement at Week 4, initiate the Non-Response Protocol (see Section 08). Document the clinical decision and rationale in the patient record and Session Log.



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Directory of TMS providers across the United States



Patient Self-Assessment

tmslist.com/quiz

Online PHQ-9 & GAD-7 screener for prospective patients

Patient Tracking Spreadsheet Layout

The following tab structure provides a complete data architecture for TMS outcome tracking. Implement in Excel, Google Sheets, or your EMR's spreadsheet module. All tabs should be linked by MRN to enable cross-tab reporting and the Aggregate Dashboard.

1. Patient Roster

2. Session Log

3. Outcome Measures

4. Follow-Up Tracker

5. Aggregate Dashboard

Tab 1 — Patient Roster

MRN	Name	DOB	Diagnosis	Insurance	Auth #	Auth Start	Auth End
100421	Martinez, R.	03/14/1978	MDD, Recurrent, Mod	Blue Cross	BC-77432	01/06/2026	04/06/2026
100422	Nguyen, T.	11/22/1985	MDD, Recurrent, Sev	United	UH-99124	01/13/2026	04/13/2026
100423	Patel, A.	07/09/1962	MDD, Single Episode	Medicare	MC-55109	12/02/2025	03/02/2026
100424	Johnson, L.	02/28/1990	MDD, Recurrent, Mod	Aetna	AE-33881	12/16/2025	03/16/2026

Roster Status Logic (computed columns)

- PHQ-9 Change** = Baseline PHQ-9 minus Current PHQ-9 (positive = improvement)
- Clinical Status** = Auto-populated from outcome definitions (Section 05)
- Sessions Remaining** = Sessions Authorized minus Sessions Completed
- Auth End** = Authorization expiration date — flag in red if ≤ 14 days

Tab 2 — Session Log

Date	MRN	Treatment Protocol	Coil Placement	Intensity (% MT)	Pulses	Duration
01/06/2026	100421	High-Freq Left DLPFC	Left DLPFC (5.5cm)	120%	3,000	37
01/08/2026	100421	High-Freq Left DLPFC	Left DLPFC (5.5cm)	120%	3,000	37
01/13/2026	100422	High-Freq Left DLPFC	Left DLPFC (5.5cm)	115%	3,000	37
01/15/2026	100421	High-Freq Left DLPFC	Left DLPFC (5.5cm)	120%	3,000	37

Outcome Measures & Follow-Up Tracker Tabs TMS Patient Outcome Tracking System

Tab 3 — Outcome Measures

MRN	Measure	Wk 0	Wk 2	Wk 4	Wk 6	Post-Tx	1mo F/U	3mo F/U	6mo F/U	Res
100421	PHQ-9	19	—	11	9	8	6	5	—	YE
100421	GAD-7	14	—	9	7	6	5	4	—	
100422	PHQ-9	19	—	17	16	14	—	—	—	PA
100422	GAD-7	12	—	11	10	9	—	—	—	
100423	PHQ-9	20	—	10	6	4	3	2	—	YE
100423	GAD-7	13	—	7	4	3	2	2	—	
100424	PHQ-9	19	—	17	18	16	—	—	—	NO
100424	GAD-7	11	—	10	11	10	—	—	—	

Response and Remission status are computed at the Post-Treatment timepoint (Week 7–9). Enter the status once per patient (not per measure row). Response: $\geq 50\%$ PHQ-9 improvement from baseline. Remission: PHQ-9 < 5 at post-treatment. See Section 05 for full definitions.

Tab 4 — Follow-Up Tracker

MRN	Name	Last Contact	1mo Due	3mo Due	6mo Due	PHQ-9 Due	Contacted
100421	Martinez, R.	01/20/2026	02/10/2026	04/10/2026	07/10/2026	02/10/2026	YES
100422	Nguyen, T.	01/15/2026	02/12/2026	04/12/2026	07/12/2026	02/12/2026	PENDING
100423	Patel, A.	01/18/2026	02/15/2026	04/15/2026	07/15/2026	02/15/2026	YES
100424	Johnson, L.	01/10/2026	02/07/2026	04/07/2026	07/07/2026	02/07/2026	NO — UTL

Follow-Up Status Codes

- **Contacted?** — Confirmed verbal or electronic contact with patient
- **Appt Booked?** — Follow-up appointment confirmed in scheduling system
- **UTL (Unable to Locate)** — 3+ contact attempts without success; initiate provider letter protocol
- **PHQ-9 Due** — Patient-reminded or mailed self-report form; enter score when returned
- **Due dates** — Calculated from last treatment session date; update automatically via formula

Tab 5 — Aggregate Dashboard (read-only summary)



Response & Remission Definitions

TMS Patient Outcome Tracking System

Standardized outcome definitions enable meaningful comparison across patients, sites, and published literature. The following thresholds are consistent with FDA clinical trial endpoints and consensus guidelines from the Clinical TMS Society.

Category	Definition	PHQ-9 Criteria	CGI-I Criteria	Clinical Implication
No Response	< 25% PHQ-9 improvement from baseline	< 25% decrease	CGI-I ≥ 4	Initiate Non-Response Protocol at Week 4. Consider protocol modification, switching to bilateral or right-sided DLPFC, or discontinuing TMS.
Partial Response	25 – 49% PHQ-9 improvement from baseline	25–49% decrease	CGI-I 3	Continue current protocol. Assess at Week 6. Consider augmentation (e.g., pharmacotherapy adjustment). Document decision rationale.
Response	$\geq 50\%$ PHQ-9 improvement from baseline	$\geq 50\%$ decrease	CGI-I ≤ 2	Continue full course. Plan taper and follow-up schedule. Discuss maintenance TMS if clinically indicated.
Remission	PHQ-9 score < 5 at post-treatment	Score < 5	CGI-I ≤ 2 (typical)	Optimal outcome. Establish relapse prevention plan. 6-month follow-up schedule. Consider maintenance TMS.
Clinical Response	Both CGI-I ≤ 2 AND $\geq 50\%$ PHQ-9 improvement (strictest standard)	$\geq 50\%$ decrease	CGI-I ≤ 2	Gold-standard endpoint for clinical trials and peer-to-peer payer reviews. Most defensible criterion for insurance authorization.

Adverse Event Tracking

All adverse events — regardless of perceived severity — must be documented in the Session Log and the patient record. Serious adverse events (SAEs) require immediate physician notification and FDA MedWatch reporting within 15 days.

Seizure Involuntary convulsive activity during or within minutes of stimulation. Requires immediate protocol halt. Always — SAE, reportable	Syncope Loss of consciousness, with or without hypotension. May accompany seizure or occur independently. Usually — SAE, reportable	Headache Most common AE. Usually mild, transient, resolves with OTC analgesics. Escalation to moderate/severe requires intervention and documentation. Mild–Moderate — intervention as needed
Pain at Stimulation Site Scalp discomfort localized to coil position. Often relates to coil positioning, intensity above 110% MT, or pulse frequency. Mild–Moderate — adjust parameters	Anxiety / Nervousness Pre-session anticipatory anxiety; may also occur during stimulation. Usually self-limited. Persistent anxiety may indicate need for pretreatment anxiolytic. Mild — reassess, monitor	Hearing Change / Tinnitus Rare. Associated with acoustic artifact from coil discharge. Should not occur with proper earplug use (hearing protection mandatory). Moderate — investigate hearing protection

Required Adverse Event Documentation Fields

Field	Description	Required For
Date & Time	Exact timestamp of event occurrence	All AEs
Event Description	Narrative description of what occurred, duration, patient-reported symptoms	All AEs
Treatment Provided	Immediate clinical response — observation, medication, transport, protocol modification	Moderate + Severe
Outcome	Resolution status — resolved, ongoing, escalated to higher level of care	All AEs
FDA Reported?	MedWatch submission confirmation number or "not applicable — not SAE"	Severe (SAE) only
Physician Notified	Name of supervising physician and time of notification	Moderate + Severe

Aggregate Reporting Template

TMS Patient Outcome Tracking System

Aggregate outcome reports serve dual purposes: internal quality monitoring and external stakeholder communication. Produce monthly reports for internal QI review and quarterly reports for referring physicians, payers, and regulatory bodies.

Monthly Outcomes Summary — [Clinic Name] — [Month/Year]

Total patients who began TMS treatment this month _____

Patients who completed full authorized course _____

Patients discontinued early (with reason) _____

Response rate (% patients meeting Response criteria) _____ %

Remission rate (% patients meeting Remission criteria) _____ %

Average PHQ-9 change among responders (baseline → post-tx) _____ points

Non-response rate (triggering Non-Response Protocol) _____ %

Number of adverse events (by severity) Mild: ____ Mod: ____ Severe: ____

Adverse event rate (AEs per 100 sessions) _____

Mean treatment sessions completed _____ / authorized

1-month follow-up completion rate _____ %

3-month follow-up completion rate _____ %

6-month follow-up completion rate _____ %

Quarterly Report — Additional Sections

Referral Source Breakdown

Psychiatry (internal) _____ %

Psychiatry (external) _____ %

Primary Care _____ %

Patient / Self-Referral _____ %

Other _____ %

Payer / Insurance Breakdown

Medicare _____ %

Medicaid _____ %

Commercial _____ %

Self-Pay _____ %

Adverse Events Summary

Total AEs this quarter: _____

SAEs (MedWatch submitted): _____

AEs by category: _____

Equipment Utilization

Total available session slots: _____

Slots utilized: _____ (____%)

Equipment downtime (hours): _____

Using aggregate data for referring physicians: Summarize de-identified outcomes quarterly. A one-page referral report can include: total patients treated, response rate, remission rate, and average PHQ-9 improvement. This builds referrer confidence and drives referral volume. Ensure no individual patient can be identified from the shared data.

For payer reports: When submitting outcome data to insurance medical directors, include: authorization period, sessions completed, PHQ-9 at baseline and each assessment point, clinical response status, and CGI-I scores. Frame remission as "PHQ-9 below clinical threshold (<5)" for clarity. Attach peer-reviewed literature citations supporting TMS efficacy if needed.

A structured QI workflow ensures consistent clinical standards, enables early identification of protocol deviations, and creates a documented framework for continuous improvement. All QI activities should be reviewed at monthly staff meetings and summarized in quarterly aggregate reports.

Monthly Chart Audit Checklist

Per-Chart Review Items (audit 10% of active charts monthly)

- | | |
|--|--|
| ☐ Baseline PHQ-9, GAD-7, CGI-S, EQ-5D-5L all documented in chart | ☐ Week 4 PHQ-9 and CGI-I completed and reviewed |
| ☐ Non-response protocol initiated if <25% PHQ-9 improvement at Week 4 | ☐ Adverse events logged in Session Log with severity classification |
| ☐ Authorization number and expiration date in patient record | ☐ Clinical status updated in Patient Roster tab |
| ☐ Follow-up appointments scheduled before treatment course completion | ☐ Informed consent for TMS updated within past 12 months |
| ☐ Motor threshold documented and re-verified if >10 days between sessions | ☐ Treatment intensity (% MT) within authorized range |
| ☐ Coil placement documented for each session | ☐ Post-treatment assessment completed within Week 7-9 window |

Non-Response Protocol — Week 4 Decision Tree

Initiate when Week 4 PHQ-9 shows <25% improvement from baseline OR CGI-I $\geq$ 4.

- 1 Review Assessment Data Accuracy**
Confirm PHQ-9 was completed correctly, patient understood items, and score was entered accurately. Rule out measurement error before changing treatment.
 - 2 Review Treatment Delivery**
Audit Session Log: verify motor threshold accuracy, coil placement, pulse count, intensity (% MT), and duration. Check for missed sessions or prolonged gaps.
 - 3 Clinical Re-Evaluation**
Prescriber reviews diagnosis, comorbidities, current medications, and psychosocial factors. Consider whether patient is taking TMS-compatible medications (no contraindications). Document evaluation in chart note.
 - 4 Protocol Adjustment (choose one or combine)**
 - A) Increase intensity:** Raise to 125-130% MT if tolerated (max per device guidelines)
 - B) Increase pulses per session:** Increase to 4,000-6,000 pulses if duration allows
 - C) Change target:** Consider switching to right-sided DLPFC (for anxious patients) or bilateral stimulation
 - D) Extend course:** Request additional authorized sessions from payer with clinical justification
 - 5 Reassess at Week 6**
Administer PHQ-9, GAD-7, and CGI-I at Week 6. Document trajectory. If still non-responsive, escalate to clinical director for case conference and consider discontinuation or referral for alternative treatment (ECT, ketamine, medication adjustment).
-

Protocol Adjustment Reference

Parameter	Standard Value	Adjustment Range	Clinical Rationale
Stimulation Intensity	100–120% Motor Threshold	Up to 130% MT (with justification)	Higher intensity increases cortical activation; most evidence-based adjustment for partial responders
Pulses per Session	3,000 pulses (left DLPFC)	Up to 6,000 pulses (accelerated protocols)	Higher pulse count may increase efficacy; monitor for increased AEs (headache, pain)
Treatment Frequency	5 sessions/week (standard)	5 sessions/week (do not reduce); consider accelerated 2x/day with 45+ min gap	Reducing below 5x/week reduces efficacy. Accelerated protocols are emerging evidence
Coil Placement	Left DLPFC (5.5cm rule)	Switch to right DLPFC (for anxious patients) or bilateral	Right-sided DLPFC may better address anxiety; bilateral may improve efficacy in severe cases
Pulse Width	Standard (4-6ms biphasic)	iTBS protocol (3-pulse bursts at 50Hz)	iTBS allows shorter session duration with comparable outcomes in some studies

Glossary of Terms

CGI-I — Clinical Global Impression – Improvement. A 7-point clinician-rated scale measuring change from baseline illness severity.

Clinical Significance — A change large enough to have practical importance for the patient or clinician, beyond statistical significance alone.

DSM-5 — Diagnostic and Statistical Manual of Mental Disorders, 5th Edition. The standard diagnostic classification system used in the United States.

GAD-7 — Generalized Anxiety Disorder 7-item scale. A validated self-report tool for screening and measuring severity of anxiety.

MADRS — Montgomery-Asberg Depression Rating Scale. A 10-item clinician-rated scale for assessing depression severity; more sensitive to change than PHQ-9.

Motor Threshold (MT) — The minimum stimulus intensity required to produce a motor evoked potential (usually in the hand muscle). Used to calibrate TMS dosing.

PHQ-9 — Patient Health Questionnaire-9. A 9-item validated self-report instrument for screening, diagnosing, and monitoring depression severity.

Remission — The absence of clinically significant depressive symptoms; in this framework, defined as PHQ-9 < 5 post-treatment.

CGI-S — Clinical Global Impression – Severity. A 7-point clinician-rated scale measuring absolute illness severity at a single timepoint.

DLPFC — Dorsolateral Prefrontal Cortex. The primary TMS target for major depression; located above and lateral to the orbit.

EQ-5D-5L — EuroQol 5-Dimension 5-Level questionnaire. A standardized quality-of-life instrument measuring mobility, self-care, activities, pain, and anxiety.

iTBS — Intermittent Theta Burst Stimulation. An accelerated TMS protocol using 3-pulse bursts at 50Hz, delivered in 2-second trains with 8-second intervals.

MDD — Major Depressive Disorder. The primary diagnosis for most TMS-treated patients; defined by DSM-5 criteria.

NIMH — National Institute of Mental Health. The U.S. federal agency funding psychiatric research; its clinical trial methodology informs TMS outcome standards.

Psychometric — Relating to the measurement of psychological variables. Psychometric properties include reliability (consistency) and validity (accuracy).

Response — Clinically meaningful improvement; in this framework, defined as $\geq 50\%$ decrease in PHQ-9 from baseline AND CGI-I ≤ 2 .

ROC Curve — Receiver Operating Characteristic curve. A plot of sensitivity against specificity used to evaluate the diagnostic accuracy of a measure at different thresholds.

Sensitivity — The ability of a test to correctly identify those with a condition (true positive rate). High sensitivity minimizes false negatives.

Statistical Significance — A result unlikely to have occurred by chance alone, conventionally at $p < 0.05$. Does not necessarily imply clinical importance.

SAE — Serious Adverse Event. Any adverse event that results in death, life-threatening injury, hospitalization, or persistent disability; requires FDA MedWatch reporting.

Specificity — The ability of a test to correctly identify those without a condition (true negative rate). High specificity minimizes false positives.

TMS Protocol — A defined set of TMS stimulation parameters (frequency, intensity, site, pulses, sessions) used to treat a specific condition.



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Online PHQ-9 and GAD-7 screener for patients

Clinical Outcome Tracking Schedule — Tear-Out Reference Card

TMS Patient Outcome Tracking System

This page is formatted as a tear-out clinical reference card. Print on cardstock and post in the treatment room or include in patient intake packets.

Clinical Outcome Tracking Schedule

TMS Treatment Timeline — Required Assessment Timepoints

	Wk 0	Wk 2	Wk 4	Wk 6	Wk 7–9	1mo	3mo	6mo
PHQ-9	Wk 0	Wk 2	Wk 4	Wk 6	Wk 7–9	1mo	3mo	6mo
	Baseline	—	Required	Required	Required	Required	Required	Required
GAD-7	Wk 0	Wk 2	Wk 4	Wk 6	Wk 7–9	1mo	3mo	6mo
	Baseline	—	Required	Required	Required	Required	Required	Required
CGI-S	Wk 0	Wk 2	Wk 4	Wk 6	Wk 7–9	1mo	3mo	6mo
	Baseline	N/A	N/A	N/A	Required	Required	Required	Required
CGI-I	Wk 0	Wk 2	Wk 4	Wk 6	Wk 7–9	1mo	3mo	6mo
	N/A	Required	Required	Required	Required	N/A	N/A	N/A
EQ-5D-5L	Wk 0	Wk 2	Wk 4	Wk 6	Wk 7–9	1mo	3mo	6mo
	Baseline	—	—	—	Required	—	—	—
Satisfaction	Wk 0	Wk 2	Wk 4	Wk 6	Wk 7–9	1mo	3mo	6mo
	Expectations	—	—	—	Experience	Optional	Optional	Optional

☒ Check = Administer this measure ☐ Blank = Not scheduled ☐ N/A = Not applicable at this timepoint

Response Thresholds at a Glance

- **PHQ-9 clinical improvement:** ≥ 5 point decrease
- **Response:** $\geq 50\%$ PHQ-9 decrease + CGI-I ≤ 2
- **Remission:** PHQ-9 < 5 at post-treatment
- **Non-response:** $< 25\%$ PHQ-9 improvement at Week 4
- **GAD-7 clinical improvement:** ≥ 4 point decrease

Critical Decision Points

- **Week 2:** First efficacy check — CGI-I only
- **Week 4:** Non-response checkpoint — initiate QI protocol if $< 25\%$ PHQ-9 improvement
- **Week 6:** Post-adjustment reassessment
- **Post-Tx (Wk 7–9):** Primary efficacy endpoint — all measures
- **At discharge:** Schedule all 3 follow-up appointments before patient leaves



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Patient Self-Assessment

tmslist.com/quiz

TMS List — tmslist.com — The national directory for TMS providers and patient resources

This reference card is intended for clinical use. Always apply clinical judgment to individual patient care decisions.

THE FREE RESOURCES LIBRARY

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Everything to choose the right TMS provider.

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Insurance Coverage Checklist

PATIENT

A step-by-step walkthrough to get TMS covered.

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TMS vs Medication Comparison

PATIENT

Outcomes, side effects and cost, side by side.

tmslist.com/downloads/tms-vs-medication.pdf →

TMS Billing & CPT Codes 2026

PROVIDER

Reimbursement reference for billers and coders.

tmslist.com/downloads/tms-billing-cpt-codes-2026.pdf →

Prior Authorization Template Kit

PROVIDER

Approval-ready letter and appeal templates.

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Patient Acquisition Playbook

PROVIDER

Channels, scripts and funnels that convert.

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Starting a TMS Clinic — Business Plan

PROVIDER

Full plan with pro-forma, equipment and staffing.

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Onboard new TMS technicians faster.

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